

****HOSPITAL DISCHARGE SUMMARY****

****PATIENT DEMOGRAPHICS****

Name: Patient 50

Age: 65 years old

Sex: Female

MRN: 000-TEST-50

Date of Admission: 06/15/2026

Date of Discharge: 06/18/2026

Attending Physician: Dr. M. Chen, MD

****PRIMARY DIAGNOSIS****

Acute appendicitis with perforation — s/p laparoscopic appendectomy

****SECONDARY DIAGNOSES****

- Rheumatoid arthritis (RA), bilateral hands and knees
- Gastroesophageal reflux disease (GERD), on chronic PPI therapy
- Hypertension, controlled
- Type 2 diabetes mellitus

****HOSPITAL COURSE****

Patient 50 presented to Metro Medical Center on 06/15/2026 with acute RLQ pain, nausea, and low-grade fever (T 38.2°C). Labs notable for WBC 14.2K (H), CRP 8.9 (H). CT abdomen/pelvis confirmed acute appendicitis with fat stranding. Patient was taken to OR on 06/15/2026 for surgical intervention.

****Operative Summary:**** Laparoscopic appendectomy performed without conversion to open. Three trocars placed (umbilical 12mm, suprapubic 5mm, RLQ 5mm). Appendix identified with inflamed, edematous appearance consistent with perforation. Appendiceal artery ligated with endoclip; appendix divided and placed in endoscopic specimen bag. Minimal free fluid noted in pelvis. No other pathology identified. Estimated blood loss <50mL. Procedure time 38 minutes. Patient tolerated anesthesia well; extubated in OR without incident.

Post-operative course uncomplicated. Pain controlled with analgesics (see below). Ambulating by POD#1. Bowel function returned POD#2 with flatus. No fever post-op; vitals stable throughout admission. Trocar sites clean, dry, intact on discharge with small occlusive dressings. Patient counseled extensively on wound care and activity restrictions prior to discharge.

****DISCHARGE MEDICATIONS****

Medication	Dose	Frequency	Duration
Acetaminophen	500 mg	q.4-6h PRN pain/fever	10 days
Ibuprofen	400 mg	q.6h PRN pain (max 1200 mg/day)	5 days
Omeprazole	20 mg	q.d., take 30 min before breakfast	Ongoing
Lisinopril	10 mg	q.d.	Ongoing
Metformin	1000 mg	BID with meals	Ongoing
Prednisone (for RA exacerbation)	5 mg	q.d.	14 days, then d/c

****No new antibiotics prescribed**** — patient completed appropriate perioperative prophylaxis (Cefoxitin).

****DIET****

Begin with ****clear liquids only**** for 24 hours post-discharge. Progress to ****bland, regular diet**** as tolerated over 48–72 hours. Avoid heavy, fatty, or spicy foods for 1 week. Continue small, frequent meals (5–6 per day). Adequate hydration essential; goal 8–10 glasses water daily. GERD precautions: no food 2–3 hours before bed.

****ACTIVITY & LIFTING RESTRICTIONS****

- ****No heavy lifting >10 lbs for 4 weeks**** post-op
- No strenuous exercise, stair climbing, or vigorous activity for 2 weeks